

Machine Translated by Google

Wed, Jul 1 at 12:03 AM I am still

working on the NDIS High Cost AT Report. It is difficult to view all at once because I have made many revisions. Could you please load the final version with the changes reflected in order into this window? If so, please start from Block 1.

Currently, the entire text of the final integrated version does not remain in this chat window, and since it contains versions that have been modified and rewritten multiple times in previous conversations, I cannot load the confirmed "Final Version Block 1" as is.

In particular, since the following modifications were added later, it is necessary to verify which version is final.

Correction of the expression 'Independence'

Revision regarding "M remains highly independent"

Reflecting low activity level

Emphasize fall risk

Reflecting muscle strength difference: 0/5 (Physiotherapist) vs 3/5 (Neurologist)

Sections 17 and 18 rewritten

NDIS Reasonable & Necessary Connection Enhancement

Complete rewriting in High Cost AT format

Claude Reconstruction based on the original structure

So the most accurate method available right now is:

Upload the latest Word file you are currently working on, or paste

Block

1 you are currently using

If you do it, I:

All modifications discussed so far reflected. 0/5 vs 3/5

strength discrepancy reflected.

Enhanced from an NDIS reviewer's perspective

Organized to a level suitable for High Cost AT submission

Therefore, I can create the final version Block 1 for actual submission.

In the current state, it is impossible to determine which of the various older versions is the latest one in use, so loading it arbitrarily may result in the latest modifications being omitted.

If you upload the latest Word file or just the current Block 1 content, I will immediately reorganize it for you in order, starting from the final version of Block 1.

NDIS_L300Go_AT_Report_Formatted.docx Document

Okay. I checked the document you just uploaded, and the version you are currently working on is a 68-page High Cost AT Report, and this is the base document we have been modifying.

To start with the review results, Block 1 (Executive Summary) currently does not yet reflect all the revisions we last agreed upon.

In particular, the following sections need correction.

Correction needed 1

Current document:

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M has lived with Multiple Sclerosis for approximately five years and experiences progressive neurological impairment affecting her mobility and functional independence.

The expression "functional independence" is included.

The direction we discussed at the end was

Emphasis on independence

ÿ Emphasis on activity restriction

ÿ Emphasis on participation restriction ÿ

Emphasis on compensatory effort ÿ

was.

Correction needed 2

Current document:

Although M remains able to mobilise using a four-wheeled walker...

This expression is fine, but

Right behind

mobility is maintained through substantial compensatory effort

While

It is better to connect the activity limitation more strongly.

Correction needed 3

Current document:

Complete loss of voluntary right ankle dorsiflexion

This part only reflects the Physio evaluation value (0/5).

What we discussed later:

Physio: DF 0/5

Neurologist: DF approximately 3/5

You must explain that the two evaluations were measured at different points in time.

The current version is a reviewer

Then why is the Neurologist 3/5?

There is a possibility that they might ask...

Block 1 based on the latest version

(Revise the entire Executive Summary into the final version for submission)

1. Executive Summary Purpose of Report

This report has been prepared to support funding approval for the purchase of a Bioness L300 Go Functional Electrical Stimulation (FES) System under the National Disability Insurance Scheme (NDIS) as High Cost Assistive Technology.

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M is a 35-year-old woman with Multiple Sclerosis (MS) who experiences significant mobility limitations associated with right-sided foot drop, lower limb weakness, gait instability, fatigue, reduced balance confidence and restricted community participation. These impairments substantially affect her ability to safely and efficiently access her home and community environments and limit her participation in meaningful daily activities.

A comprehensive assessment process was completed, including:

- Detailed orthotic and functional assessment
- In-clinic Bioness L300 Go trial
- Eight-week community-based trial
- Three-week washout period following device return
- Physiotherapy outcome measurement
- Participant and carer feedback collection
- Neurologist review and endorsement

The purpose of this report is to determine whether the Bioness L300 Go represents the most appropriate, effective and reasonable assistive technology intervention to address M's disability-related mobility limitations and support achievement of her NDIS goals.

Participant Overview

M has lived with Multiple Sclerosis for approximately five years and experiences progressive neurological impairment affecting lower limb function, walking efficiency and community mobility.

Her condition has resulted in:

- Right foot drop during swing phase
- Reduced foot clearance
- Ankle instability
- Knee hyperextension during stance
- Walking-related fatigue
- Reduced standing tolerance
- Reduced walking endurance
- Reduced balance confidence
- Activity avoidance behaviours
- Reduced participation in community, family and social activities

Although M is able to mobilise using a four-wheeled walker, this mobility is achieved through substantial compensatory effort, careful activity pacing and avoidance of situations that increase fatigue, instability or falls risk.

Importantly, M routinely limits the frequency, duration and complexity of activities she undertakes in order to manage fatigue and reduce the likelihood of tripping, near-falls and mobility-related incidents. As a result, her functional limitations extend beyond gait impairment and significantly restrict participation across multiple life domains.

Assessment Findings

Clinical assessment identified the following neurological and functional impairments.

Neurological Impairments

Right ankle dorsiflexion weakness (Physiotherapy assessment: 0/5)

Right ankle dorsiflexion strength subsequently documented by treating neurologist as approximately 3/5 during review

Right ankle eversion: 0/5

Right ankle inversion: 1/5

Right plantarflexion weakness

- Mild spasticity affecting plantar flexors
- Functional Impairments
- Foot drop during swing phase
- Toe drag
- Inversion during gait
- Knee hyperextension during stance
- Reduced walking speed
- Reduced walking endurance
- Reduced standing tolerance
- Increased fatigue with activity
- Increased risk of trips and near-falls

These impairments directly restrict M's ability to safely access her community, engage in daily activities and participate in family and social roles.

This version is what we have discussed so far

Correction of the expression 'Independence'

- Reflecting low activity level
- Emphasize fall risk

Emphasis on Activity Avoidance:

- 0/5 vs 3/5 Discrepancy Explanation
- NDIS reviewer perspective

This is the final Block 1 for actual submission that reflects that.

The next step is to compare Block 2 (Section 2 entire: Participant Profile and Disability Impact) with the current document and finalize it to reflect the revisions.

Mon, Jul 13 at 12:44 AM Fatigue-
Severity-Scale_copy.pdf PDF

Okay. I checked the FSS I just uploaded.

According to this data:

- Assessment Date:
- 21/01/2026 FSS Questions 1–9 all
- 7 points Total
- Score: 63/63 Average
- Score: 7.0/7 VAFS displayed as approximately 6/10

This effectively means the maximum level of fatigue.

Therefore, the FSS section of Appendix A can be written much more strongly than the existing version.

A10. Fatigue Severity Scale (FSS)

The Fatigue Severity Scale (FSS) was completed by the participant on 21 January 2026 and reviewed by the treating physiotherapist.

- Baseline FSS Results
- Measure Result
- Total Score 63/63
- Average Score 7.0/7

The participant scored the maximum rating of 7/7 on all nine FSS items, indicating severe fatigue affecting all measured domains of daily function.

Assessment findings demonstrated that fatigue:

Reduced motivation and activity participation

Was exacerbated by physical activity

Interfered with physical functioning

Caused ongoing difficulties in daily life

Limited sustained physical activity

Interfered with responsibilities and routine activities

Was considered one of the participant's most disabling symptoms

Negatively affected family, social and community participation

The findings indicate that fatigue represented a major disability-related barrier to mobility, endurance, participation and overall function.

Given the participant's diagnosis of Multiple Sclerosis, the severity of fatigue identified at baseline was considered clinically significant and consistent with the substantial activity restriction observed throughout assessment.

Although repeat FSS assessments were not completed during the community trial, participant reports, carer feedback and community trial observations indicated a reduction in fatigue-related activity restriction during active use of the Bioness system.

And the dates in Appendix A can now be edited as well.

A1 Assessment Overview

existing:

A comprehensive physiotherapy assessment was completed on [Insert Assessment Date]

correction:

A comprehensive physiotherapy assessment was completed on 21 January 2026 prior to commencement of the Bioness L300 Go community trial.

There is practically only one item left to check in Appendix A right now.

How to record outdoor mobility

In the initial Physio email

Independent with a 4-Wheeled Walker indoors

There is only,

We later

Community mobility is performed very cautiously. Low activity level restrictions are in place.

I added the information that...

So, to refine the A7 Mobility Status for the final submission:

If you let us know whether 4WW was used outdoors as well, or only indoors, we can finalize Appendix A.

If you look at actual NDIS High Cost AT reports, there are more cases where Orthotist review notes or Community trial review notes are sufficiently explained in the main body and only original supporting documents are attached, rather than including them all as an Appendix.

Your case is already:

Block 5 (Clinical Assessment Findings)

Block 8 (In-Clinic Trial Outcomes)

Block 9 (Community Trial Methodology)

Block 10 (Community Trial Outcomes)

Block 14 (Washout Findings)

All the content is included in it.

So, B and C can be simplified as follows.

APPENDIX A – PHYSIOTHERAPY ASSESSMENT AND OUTCOME MEASURES

Attachment A1

Physiotherapy Referral and Clinical Assessment Summary

Attachment A2

Physiotherapy Outcome Measure Summary Table

Attachment A3

Fatigue Severity Scale (FSS)

Attachment A4

Patient Specific Functional Scale (PSFS)

APPENDIX B – ORTHOTIC ASSESSMENT AND CLINICAL EVALUATION

Attachment B1

Orthotic Assessment Record

Attachment B2

Bioness L300 Go Configuration Record

(One orthotic assessment form or fitting form)

Purpose

To provide supporting clinical documentation relating to orthotic assessment, device selection and clinical evaluation of Functional Electrical Stimulation.

APPENDIX C – COMMUNITY TRIAL EVIDENCE

Attachment C1

Participant Community Trial Logbook Summary

(Can be deleted if not found)

or

Attachment C1

(Page 1)

Date Activity

Initial fitting Commencement of trial

8 Apr Review

21 Apr Review and optimisation

11 May Final review and device return

Washout 3 weeks

But honestly, this isn't really necessary either.

If I actually submit it

Appendix B

Attachment B1

Orthotic Assessment Form

(The actual orthotic assessment you wrote)

Appendix C

delete

because

Community Trial proof is already:

Appendix D 8-

week Logbook

Washout Logbook

Appendix E

Participant Feedback

Appendix F

Mother Feedback

Appendix G

Neurologist Letter

Appendix H

Product Brochure

Quote

It's because it's all included in it.

The cleanest structure I recommend for the final submission

Appendix A

Physiotherapy Assessment and Outcome Measures

Referral/Assessment Summary

Outcome Measure Table

FSS

PSFS

Appendix B

Orthotic Assessment Record

Appendix C

Participant Logbooks

8 Week Trial Logbook

3 Week Washout Logbook

Appendix D

Participant Feedback

Appendix E

Carer Feedback

Appendix F

Neurologist Review Letter

Appendix G

Product Brochure and Quote

I believe this structure is the cleanest configuration, allowing you to use only the materials you actually have right now without having to create new separate "note" documents for B and C. From the examiner's perspective, it makes it easy to find the necessary evidence and minimizes duplication.

Please find a way to replace the physio referral or assessment summary as well. I don't think there is a need to duplicate the content already included in the report itself in the appendix.

Looking at the overall structure again now, I think I tried to create too many Appendix.

From the perspective of an NDIS High Cost AT examiner:

The examiner

Verify text

reading results

Verify supporting documents

Look at it in order.

Already in your text (Blocks 1~20)

Physiotherapy findings

Strength

ROM

Spasticity

Gait findings

Outcome measures

Trial methodology

Trial outcomes

Washout findings

All of this is included,

If you attach a document in the Appendix that explains the same content again

"Why did you write the same thing twice?"

Become.

If I actually submit it

Appendix A can also be deleted

Already included in the text

Section 5 Clinical Assessment Findings

Section 8 In-Clinic Trial Outcomes

Section 10 Community Trial Outcomes

Section 14 Washout Findings

So, specifically

Physiotherapy Referral

Assessment Summary

There is no need to include the same "document I rewrote" as an Appendix.

Keep only the truly necessary Appendix

Appendix A – Outcome Measure Evidence

Attachment A1

Physiotherapy Outcome Measure Table

(Original Physio Email)

Attachment A2

Fatigue Severity Scale (FSS)

Attachment A3

Patient Specific Functional Scale (PSFS)

Why are you leaving this behind?

Although numbers are included in the text

The examiner

Show me the original data

Needed when doing.

Appendix B – Participant Trial Documentation

Attachment B1

8 Week Community Trial Logbook

Attachment B2

3 Week Washout Logbook

This is also original proof.

Appendix C – Participant Feedback

Attachment C1

Participant Feedback Statement

(Page 1 of the Quotes you compiled)

Mother / Primary Carer Feedback Statement

Appendix E – Medical Evidence
Attachment E1

Neurologist Letter

(If a letter of support is included, this one is enough)

Appendix F – Product Information and Quote
Attachment F1

Bioness L300 Go Product Brochure

Attachment F2

Supplier Quote

The final structure I recommend

Section 1 ~ 20

(Includes all clinical explanations)

Appendix A

Outcome Measure Evidence

Appendix B

Trial Logbooks

Appendix C

Participant Feedback

Appendix D

Carer Feedback

Appendix E

Neurologist Evidence

Appendix F

Product Brochure & Quote

The advantages of this structure are:

• Almost no overlap

• Examiners can immediately verify only the original proof

• Reduce page count by 10~15 pages or more

• The roles of "reports" and "supporting documents" are clearly separated.

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Actually, if it is a High Cost AT report with sufficient data like your case, I would submit it by omitting all separate appendix documents such as the Physiotherapy Referral Summary, Orthotic Assessment Summary, and Community Trial Review Summary, and including only the original supporting documents in the appendix. This looks the cleanest and most professional.

Please write it so that the appendix can be copied and pasted by only including original supporting documents.

The version below is the Appendix structure I would actually use for the final submission of the NDIS High Cost AT Report.

The core principle is:

Main Text (Sections 1–20) = Clinical Analysis, Interpretation, Evidence

Appendices = Attach only original supporting documents

In other words, it is a method where I attach only the actual supporting documents without putting the Summary or Reasoning documents I wrote back into the Appendix.

21. APPENDICES

The following appendices contain supporting evidence referenced throughout this report.

The appendices have been limited to original supporting documentation, objective outcome measure data, participant records, medical evidence and supplier information. Clinical interpretation, analysis and recommendations are contained within the body of this report.

Appendix A – Outcome Measure Evidence
Attachment A1

Physiotherapy Outcome Measure Summary Table

(Including 10 Metre Walk Test, Timed Up and Go, Six Minute Walk Test, Dynamic Gait Index and Thirty Second Sit-to-Stand Test results completed by the treating physiotherapist)

Attachment A2

Fatigue Severity Scale (FSS)

Attachment A3

Patient Specific Functional Scale (PSFS)

Purpose

To provide objective outcome measure data collected throughout the assessment process.

Appendix B – Community Trial Documentation
Attachment B1

Eight-Week Community Trial Logbook

Attachment B2

Three-Week Washout Logbook

Purpose

To provide participant-recorded information regarding device use, daily activities, mobility experiences, fatigue, confidence and functional performance throughout the community trial and washout periods.

Appendix C – Participant Feedback
Attachment C1

Purpose

To provide participant-reported experiences regarding the impact of the Bioness L300 Go System on mobility, participation, confidence and quality of life.

Appendix D – Carer Feedback

Attachment D1

Mother / Primary Carer Feedback Statement

Purpose

To provide independent observations regarding functional changes, participation and confidence observed during the community trial.

Appendix E – Medical Supporting Documentation

Attachment E1

Neurologist Clinical Review and Recommendation

Purpose

To provide supporting medical evidence regarding diagnosis, functional presentation and suitability for Functional Electrical Stimulation technology.

Appendix F – Product Information and Financial Documentation

Attachment F1

Bioness L300 Go Product Information

Attachment F2

Supplier Quotation

Purpose

To provide technical and financial information relating to the recommended Assistive Technology.

Appendix Summary

The attached appendices provide objective clinical evidence, participant-reported outcomes, carer observations, medical supporting documentation and supplier information which collectively support the recommendation for funding of the Bioness L300 Go Lower Leg and Thigh Components as a reasonable and necessary Assistive Technology support under the NDIS.

If it is this version:

Physiotherapy Referral Summary [ÿ ÿÿ](#)

Orthotic Assessment Summary [ÿ ÿÿ](#)

In-Clinic Trial Summary [ÿ ÿÿ](#)

Community Trial Plan [ÿ Delete](#)

Review Summary [ÿ Delete](#)

Device Optimization Summary [ÿ Delete](#)

Washout Summary [ÿ Delete](#)

